

Department of Health and Human Services
Office of Inspector General



Office of Evaluation and Inspections

DATA BRIEF

June 2026 | OEI-09-24-00331

**Medicare Advantage Organizations
Overturned Nearly All Appealed
Prior Authorization Denials for
Skilled Nursing Facility Admission,
Raising Concerns About Initial
Denials**



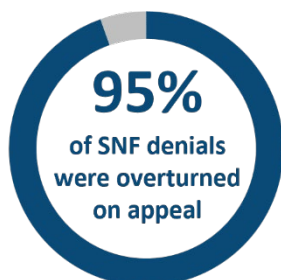
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Medicare Advantage Organizations Overturned Nearly All Appealed Prior Authorization Denials for Skilled Nursing Facility Admission, Raising Concerns About Initial Denials

Why OIG Did This Review

- [Previous OIG work](#) raised concerns that Medicare Advantage organizations' (MAOs') use of prior authorization can in some cases result in denials and delays in access to needed care for enrollees. MAOs that inappropriately deny care are not delivering the full value that taxpayers pay them to provide.
- OIG identified denials of prior authorization requests for post-acute care after a hospital stay as a particular area of concern. This report highlights new analyses of denial and overturn rates of requests for admission to skilled nursing facilities (SNFs), which provide short-term skilled nursing and/or therapy services to help patients recovering from an illness, injury, or surgery.

What OIG Found



- In June 2024, the 19 MAOs in this review collectively denied 12 percent of requests for SNF admission. MAO denial rates ranged from 23 percent to 0.4 percent.
- Enrollees and their providers appealed 18 percent of SNF denials. When SNF denials were appealed, MAOs overturned 95 percent in favor of the enrollee. The extremely high overturn rate indicates that some enrollees were initially denied medically necessary care and raises concerns about denials that were not appealed.
- The MAO contractor naviHealth processed half of all requests for SNF admission and denied 14 percent of them—a higher denial rate than MAOs that processed requests internally (11 percent) and other contractors (9 percent). MAOs later overturned 97 percent of SNF denials issued by naviHealth when enrollees appealed. This raises concerns about whether contractors are receiving appropriate training and oversight from MAOs. NaviHealth is a subsidiary of the MAO United Health Group, Inc.
- MAOs and their contractors denied requests for SNF-level care from nursing home residents 40 percent of the time, a much higher rate than requests from all other enrollees (11 percent).

What OIG Recommends

To identify and respond to concerning patterns of prior authorization denials, [CMS](#) should regularly collect request-level prior authorization data that include service type and contractor information (see our companion report, OEI-09-24-00330). Further, CMS should: (1) take action to address any breakdowns in the initial reviews of SNF admission requests that are driving the extremely high overturn rate of SNF admission denials, (2) assess reasons for variation in SNF denial rates across MAOs and contractors and take action as appropriate, and (3) assess reasons for the differences in SNF denial rates between nursing homes and non-nursing home residents and take action as warranted. CMS did not explicitly concur or nonconcur with our three recommendations.

Primer: Medicare Advantage and Post-Acute Care

Medicare Advantage

People who are eligible for benefits under original Medicare may choose to receive their Medicare coverage through Medicare Advantage plans administered by Medicare Advantage organizations (MAOs). In 2024, 54 percent of people with Medicare (32.8 million people) were enrolled in Medicare Advantage, and the Centers for Medicare & Medicaid Services (CMS) paid MAOs an estimated \$494 billion.^{1, 2}

Medicare Advantage plans must cover at least the same services as original Medicare.³ MAOs are expected to implement critical program controls to avoid unnecessary costs and ensure program integrity while at the same time ensuring that enrollees can get necessary and appropriate care. To do that, MAOs may impose additional administrative requirements, such as requiring prior authorization before certain services can be provided. For example, MAOs may require prior authorization for services with a history of improper payments in original Medicare, such as requests for skilled nursing facility care.⁴

Under Medicare Advantage's payment model, MAOs are paid a risk-adjusted, capitated payment each month for each enrollee regardless of the number of services the enrollee receives.⁵ A central concern about capitated payment models is the potential incentive for insurers to deny enrollees' access to services in an attempt to increase profits. Inappropriate denials of care can have serious impacts for enrollees' health, and MAOs that inappropriately deny care are not delivering the full value that taxpayers pay them to provide.

Facility-Based Post-Acute Care for Medicare Advantage Enrollees

Post-acute care facilities provide medical, therapeutic, and rehabilitative care to patients, typically after a qualifying stay in a general acute-care hospital, to help patients regain the ability to return to their home or community setting.⁶ For patients who need post-acute care, the hospital care team typically helps to identify which post-acute care setting is appropriate for them. After considering patient needs and preferences, the hospital submits a request for admission to the selected facility.⁷ The post-acute care facility then conducts its own assessment to determine whether the patient meets its admission criteria. Post-acute care facilities include long-term care hospitals (LTCHs),⁸ inpatient rehabilitation facilities (IRFs),⁹ and skilled nursing facilities (SNFs).^{10, 11}

Long-term care hospitals

(LTCHs) provide acute care to patients who need hospital-level care for extended periods of time. LTCHs provide specialized services such as respiratory therapy, head trauma treatment, and pain management.

Inpatient rehabilitation

facilities (IRFs) provide intensive therapy and medical services in a hospital environment to patients with complex nursing, medical management, and rehabilitative needs.

Skilled nursing facilities

(SNFs) provide short-term skilled nursing or therapy services to patients for rehabilitation following inpatient hospital stays, such as for joint replacement or stroke.

If a post-acute care facility approves a patient for admission, and the patient's MAO requires prior authorization for post-acute care, then the facility or the hospital care team must request authorization for the post-acute care stay from the MAO. The MAO reviews prior authorization requests to determine whether to approve or deny the request; this review can occur internally in the MAO or the MAO may contract with another company to conduct the review.¹² If the MAO or its contractor denies the request for post-acute care, the patient or their care team can appeal the denial back to the MAO.¹³

Nursing Homes and Skilled Nursing Care

Nursing homes typically operate as both **skilled nursing facilities (SNFs)**, where they provide short-term skilled care following a hospitalization, and as **nursing facilities**, where they provide lower intensity routine nursing care (often referred to as “custodial care”) for individuals with functional and/or cognitive impairments. Medicare pays for short-term SNF care after a hospitalization for enrollees who require skilled nursing care and meet other Medicare coverage requirements (as described above). Medicare does not pay for room and board services, routine nursing care, or assistance with activities of daily living for nursing facility residents (i.e., those staying longer than 100 days). Those services are largely covered by Medicaid, other private insurance plans, or residents’ out-of-pocket payments. Nursing home residents sometimes shift back and forth between receiving short-term SNF-level care following a hospital stay, and once discharged from that care, receiving less intensive long-term care.¹⁴

Related Work

This report is part of a body of work that the Office of Inspector General (OIG) has conducted related to denials of care in managed care programs. This report also has a companion report, OEI-09-24-00330, which analyzes MAO data on prior authorization requests for LTCH and IRF stays. In future work, OIG will conduct an in-depth review of a sample of case files to examine MAOs’ processes for reviewing prior authorization requests for post-acute care.

In a 2022 report, OIG physician reviewers found that among the prior authorization requests (for all service types) that MAOs denied, 13 percent met Medicare coverage rules.¹⁵ In other words, these services likely would have been approved if the patients had been enrolled in original Medicare rather than in Medicare Advantage. One of the prominent service types that was denied even though the requests met Medicare coverage rules was requests for post-acute care. Denying requests that meet Medicare coverage rules may prevent or delay enrollees from receiving medically necessary care and can burden providers.

In a 2018 report, OIG found that when enrollees and providers appealed denied requests between 2014 and 2016, MAOs overturned about 75 percent of their own prior authorization and payment denials.¹⁶ This indicates that at least some of these denials were inappropriate or could have been avoided, given that ultimately the plans agreed to authorize the medically necessary services and make the payments. OIG also found that CMS cited more than half of audited Medicare Advantage contracts in 2015 for inappropriately denying prior authorization and payment requests.

OIG has also conducted work related to denials of care and payment in Medicaid managed care and Medicare Part D.^{17, 18, 19, 20}

How OIG Did This Review

To calculate denial and appeal rates for SNF admission requests, we collected data from the 19 largest MAO parent companies by enrollment about the prior authorization requests that they or their contractors processed in June 2024. To resolve any discrepancies in the data and to better understand the trends in our analysis, we followed up in writing with select MAOs. We also interviewed patient advocacy and industry organizations to hear their perspectives. The analysis in this report provides important insights about prior authorization patterns across companies and indicates opportunities for followup and potential action, but we cannot determine from this data analysis alone whether or how many of these denials were inappropriate.

The 19 MAOs in this study had 29.3 million people enrolled in their Medicare Advantage contracts in June 2024, which represented 86 percent of enrollment in the Medicare Advantage program at that time (see Appendix A).

FINDINGS

Overall, MAOs denied 12 percent of requests for SNF admission; denial rates ranged from 23 percent to 0.4 percent

In June 2024, the 19 MAOs in this review denied around 13,500 of 109,400 requests for SNF admission (12 percent). Denial rates for SNF admission requests varied among the 19 MAOs with some MAOs denying requests at much higher rates than their peers (see Appendix B for MAO request volume and denial rates).

Exhibit 1: Humana Inc.; CVS Health Corporation; and UnitedHealth Group, Inc., received the most requests for SNF admission and had three of the five highest denial rates

MAO	SNF requests June 2024	SNF denial rate
Molina Healthcare, Inc.	591	23.4%
Kaiser Foundation Health Plan, Inc.	1,510	14.0%
Humana Inc.	26,450	13.5%
CVS Health Corporation	16,108	13.5%
UnitedHealth Group, Inc.	41,486	12.9%
Devoted Health, Inc.	892	11.1%
Elevance Health, Inc.	7,261	10.8%
Centene Corporation	2,750	10.6%
BCBS of Michigan Mutual Ins. Co.	2,577	9.4%
The Cigna Group	2,705	8.8%
Highmark Health	1,922	8.7%
Health Care Service Corporation	745	6.7%
GuideWell Mutual Holding Corporation	583	5.1%
Aware Integrated, Inc.	648	4.3%
Corewell Health	685	3.9%
Healthfirst, Inc.	748	3.3%
SCAN Group	257	1.6%
UPMC Health System	1,227	1.4%
MHH Healthcare, L.P.	242	0.4%

Source: OIG analysis of June 2024 SNF prior authorization data for MAOs that received at least 50 SNF requests.

Note: CVS Health Corporation is the parent company of Aetna.

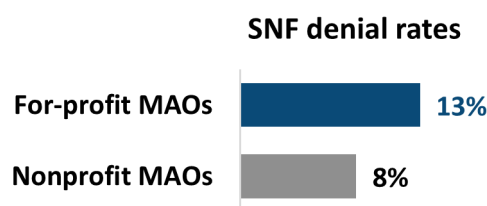
The three largest MAOs by enrollment (UnitedHealth Group, Inc.; Humana Inc.; and CVS Health Corporation) received three-quarters of the SNF requests and denied them at some of the highest rates. Nearly 20 million people were enrolled in plans operated by these 3 companies.²¹ (See Appendix A for information on MAO enrollment.) In addition to the three largest MAOs, some of the smaller MAOs also had higher denial rates, but they reviewed far fewer SNF cases. For example, Molina Healthcare, Inc., denied 23 percent of requests for SNF admission—nearly double the overall SNF denial rate.

Although no specific denial rate is expected or correct, the wide variation in MAO denial rates for SNF requests is concerning because it is unclear why some MAOs had denial rates that were much higher than their peers. Extremely high or low denial rates may indicate differences in MAO policies or performance, such as how they interpret or apply coverage criteria. In future work, OIG will conduct an in-depth review of a sample of case files to examine MAOs' processes for reviewing prior authorization requests for post-acute care.²²

Denial rates for SNF admission were higher among for-profit MAO contracts than nonprofit MAO contracts

Differences in denial rates between for-profit and nonprofit MAO contracts suggest that financial incentives may be partially driving higher denial rates among some MAOs (see Appendix E for details). MAOs can operate for-profit contracts only, nonprofit contracts only, or both types of contracts. The MAO with the highest SNF denial rate, Molina Healthcare, Inc., operated only for-profit contracts at the time of our review, as did the three largest MAOs. (See Appendix A for the number of for-profit and nonprofit contracts operated by each MAO.)

Exhibit 2: For-profit MAO contracts denied SNF admission requests at a higher rate than nonprofit contracts



Source: OIG analysis of June 2024 MAO SNF prior authorization data and 2024 contract tax status from CMS's Health Plan Management System.

MAOs have a financial incentive to deny SNF-level care and instead approve a lower level of care, such as home health services or outpatient therapy. Although information about what MAOs pay to different types of providers is not available, payment amounts in original Medicare can provide helpful context for the relative

cost of care in each setting. The average cost of a SNF stay in original Medicare in 2023 was \$16,000, while average home health care costs were only \$6,000.^{23, 24}

MAOs overturned nearly all denials for SNF admission when enrollees appealed, indicating that some enrollees were initially denied medically necessary care

18% of SNF denials were appealed

The appeals process is an important safeguard to help ensure that patients have access to needed services. Enrollees and providers appealed around one-fifth of requests for SNF admission that MAOs denied in June 2024 (approximately 2,400 of 13,500 denials). For the 82 percent of denials that were not appealed, the patient may have requested a lower level of care (e.g., home health), paid for SNF-level care out of pocket, or gone without post-acute care.

Among the denials of SNF admission that enrollees appealed, the 19 MAOs in this study overturned 95 percent in favor of the enrollee (2,313 of 2,445 appeals). The high overall overturn rate was driven largely by the MAO UnitedHealth Group, Inc., which received 42 percent of the SNF appeal requests and overturned them 99.7 percent of the time. In addition to UnitedHealth Group, Inc., all of the MAOs that received at least 50 appeals had extremely high overturn rates (see Appendix B for overturn rates by MAO).

Exhibit 3: Overturn rates for denials of SNF admission were extremely high across MAOs that received at least 50 appeals

MAO	SNF appeals June 2024	Overturn rate
Elevance Health, Inc.	93	100.0%
UnitedHealth Group, Inc.	1,025	99.7%
CVS Health Corporation	382	98.2%
The Cigna Group	50	96.0%
Humana Inc.	672	92.1%
BCBS of Michigan Mutual Ins. Co.	75	90.7%

Source: OIG analysis of June 2024 SNF appeals data for MAOs that received at least 50 appeals.

Among MAOs with at least 50 SNF appeals, the overturn rate ranged from nearly 91 percent for BCBS of Michigan Mutual Insurance Co. to 100 percent for Elevance Health, Inc. Among the smaller MAOs that did not receive at least 50 SNF appeals each, the combined overturn rate was only 59.5 percent.

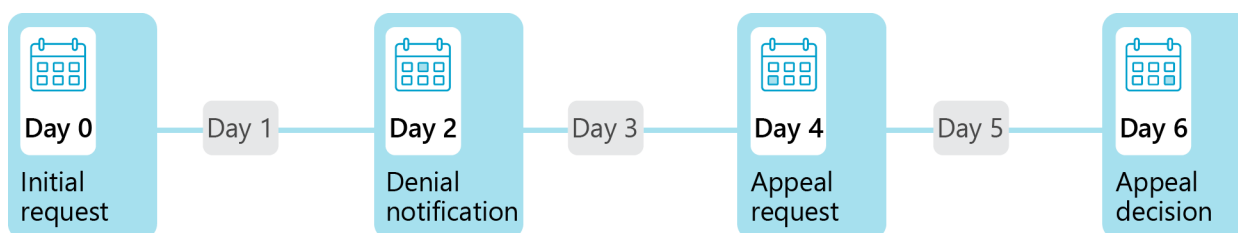
This analysis provides important insights about overturn patterns across companies and indicates opportunities for followup and potential action, but we cannot

determine from this data analysis alone whether or how many of these denials were inappropriate. However, the extremely high overturn rate of appealed SNF admission denials raises concerns that more initial SNF requests should have been approved rather than denied because the MAO ultimately determined the requested care was medically necessary. Although overturned denials mean that the enrollee was eventually approved to receive the requested care, each overturned denial represents a case in which the patient or their provider had to file an appeal to access SNF services that were medically necessary and covered by Medicare. This extra step may add unnecessary complexity and administrative burden for enrollees, providers, and MAOs, and may have delayed enrollee access to SNF care.

Enrollees who appealed SNF denials typically received a decision 6 days after their initial request, with some waiting much longer

For enrollees who appealed prior authorization denials for SNF admission, the typical (i.e., median) wait time from the initial request date to the appeal decision date was 6 days. Many enrollees waited longer to receive a final appeal decision—17 percent of SNF appeals took 10 days or longer. In contrast, when MAOs approved the initial request for SNF admission, they typically approved the request on the same day that the request was made, enabling a much faster discharge from the hospital.

Exhibit 4: Enrollees who appealed SNF admission denials typically waited 6 days to receive a final decision



Source: OIG analysis of MAO SNF appeals data for June 2024 denials. The timeline shows the median number of days between each step for denials that were appealed.

Enrollees who are requesting post-acute care, including those appealing denials, are usually receiving care in an acute-care hospital while they wait for the MAO’s decision. Other research has shown that Medicare Advantage enrollees stay in the hospital longer than enrollees with original Medicare before being discharged to a SNF.²⁵ Unnecessary or “avoidable” days spent in a hospital can mean a significant financial cost to hospitals that is not separately reimbursable.^{26, 27} Delayed discharges can also negatively affect patients’ psychological and social well-being and contribute to feelings of uncertainty and stress about their recovery and prognosis.²⁸

The MAO contractor naviHealth denied SNF requests at a higher rate than MAOs that processed requests internally; many of these denials were later overturned by MAOs

MAOs can choose to process SNF requests internally or can hire contractors to process the requests for them.²⁹ Across the 19 MAOs in our review in June 2024, 68 percent of SNF requests were processed by contractors and 32 percent were processed by MAOs internally.

The MAO contractor naviHealth denied one in seven requests for SNF admission

The contractor naviHealth (a subsidiary of the MAO UnitedHealth Group, Inc.³⁰) processed half of all requests for SNF admission among the 19 MAOs in June 2024. Most of the SNF requests that naviHealth reviewed (63 percent) were for UnitedHealth Group, Inc., but naviHealth also processed requests for BCBS of Michigan Mutual Insurance Co., Highmark Health, and Humana Inc.

Exhibit 5: The MAO contractor naviHealth denied requests for SNF admission at a higher rate than other contractors and MAOs that processed requests internally

MAO contractor	SNF requests June 2024	SNF denial rate	
naviHealth	55,176		14%
MAOs – internal	35,212		11%
Other contractors	18,999		9%

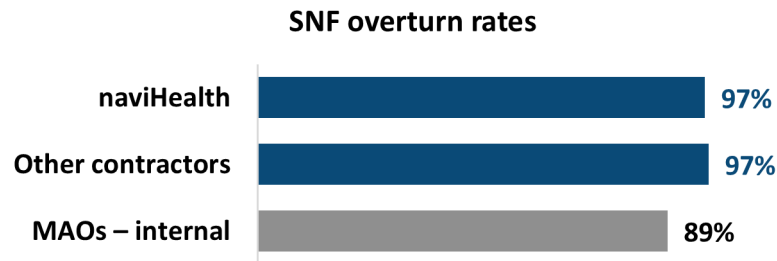
Source: OIG analysis of June 2024 MAO SNF prior authorization data.

NaviHealth denied 14 percent of requests for SNF admission, a higher rate than requests that were reviewed internally by MAOs (11 percent) or reviewed by other MAO contractors (9 percent) (see Appendix C). As found in our companion report, naviHealth also denied requests for admission to LTCHs and IRFs at higher rates than MAOs and other contractors.³¹ Variation in denial rates between contractors and MAOs that review requests internally raise concerns about whether the contractors are receiving appropriate training, guidance, and oversight from MAOs on how to assess requests.

MAOs overturned SNF requests denied by all contractors, including naviHealth, at high rates upon appeal

Although many MAOs used contractors to review initial prior authorization requests, they reviewed any appeals internally. SNF denials issued by both naviHealth and other contractors were overturned at higher rates on appeal than denials that MAOs reviewed internally.

Exhibit 6: SNF denials issued by MAO contractors were more likely to be overturned upon appeal than denials issued by MAOs



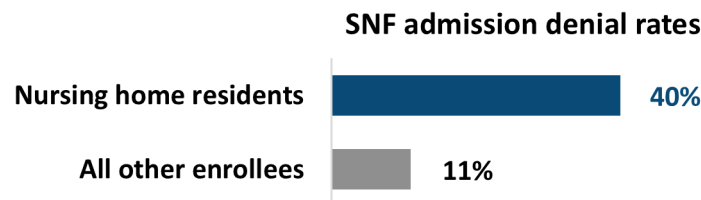
Source: OIG analysis of June 2024 MAO SNF appeals data.

Higher overturn rates of denials issued by contractors could indicate differences in policies or procedures between the parent company and its contractor or differences in interpretation of CMS coverage rules.

Nursing home residents were more likely than other enrollees to be denied admission to SNF-level care

MAOs denied 40 percent of requests for SNF admission for people who lived in nursing homes around the time of their request (i.e., “long-stay” residents with more than 100 days in a nursing home)³² but denied only 11 percent of all other requests. Nursing home residents submitted around 3 percent of requests for SNF admission to the 19 MAOs in June 2024 (approximately 3,200 out of 109,400 requests). We examined denial rates by other enrollee characteristics but noted smaller differences (see Appendix D).

Exhibit 7: Nursing home residents were more likely than other enrollees to be denied admission to SNF-level care



Source: OIG analysis of June 2024 MAO SNF prior authorization data and 2024 data on enrollee nursing home stays from the Minimum Data Set.

MAOs indicated that for enrollees who are denied SNF-level care, including nursing home residents, the MAO may authorize less costly intermittent skilled nursing services and/or outpatient therapies instead.³³ As shown in Exhibit 8, the care requirements for patients in SNFs are more frequent than for patients in long-term nursing facility care. MAOs pay for the full cost of care for patients in SNFs. For patients in long-term nursing facilities, MAOs pay for outpatient health care services only; they do not pay for room and board or other routine, non-skilled care.

Exhibit 8: Care requirements are more intensive—and MAO costs are higher—for short-term SNF care than for long-term nursing facility care

Short-term skilled nursing facility care		Long-term nursing facility care with outpatient services	
Paid by the MAO	- Room and board services - Assistance with activities of daily living - Care is planned and managed by skilled personnel such as physicians or nurses	- Room and board services - Assistance with activities of daily living - Routine nursing care (e.g., medication management, changing dressings)	Paid by Medicaid, private pay, or other insurance
	<u>Must include:</u> Daily skilled nursing care (e.g., IV feedings, intramuscular injections) and/or 5-7 days a week of inpatient skilled therapies (e.g., physical therapy, speech therapy)	<u>May include:</u> - Intermittent skilled nursing care – no daily requirement and/or - outpatient skilled therapies – no daily requirement	Paid by the MAO

Source: OIG analysis of CMS coverage rules for long-term care and SNFs.

MAOs offered explanations for why nursing home residents may be less likely than other enrollees to meet SNF coverage rules

When asked what factors might cause the SNF admission denial rate to be higher for nursing home residents than for other enrollees, some MAOs reported that nursing home residents are less likely to meet the admission criteria for daily SNF-level care because they already have some intermittent skilled therapy supports available.³⁴ Other MAOs stated that nursing home residents may have a more complex medical history than enrollees who are able to live elsewhere, including more severe or compounded physical or cognitive functional impairments. MAOs stated that a complex medical history, regardless of where the enrollee resides, may diminish the

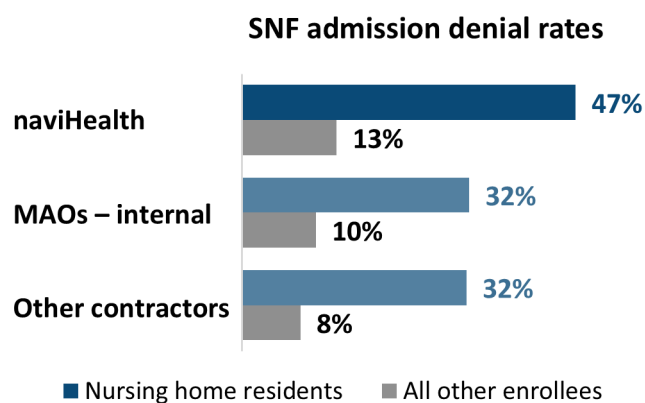
enrollee’s ability to meaningfully participate in daily skilled therapies in a SNF, and thus may lead to a higher denial rate.

Patient advocates and nursing home industry organizations stated that an enrollee’s place of residence should not affect whether they meet criteria for SNF admission

When asked about the difference in denial rates between nursing home residents and other enrollees, representatives from patient advocacy and nursing home industry groups told OIG that, in their view, an enrollee’s place of residence should not affect whether they meet the criteria for admission to a SNF. One representative stated that long-term custodial care in a nursing facility, even with outpatient therapies (e.g., physical therapy) or intermittent skilled nursing care, is not a clinically equivalent substitute for the care management and daily therapy and/or daily skilled nursing that is provided as part of SNF-level care. Another representative stated that skilled clinicians can adapt therapies for people with physical or cognitive functional impairments such as dementia, and that SNFs report that people with those impairments still benefit from those therapies.

The MAO contractor naviHealth denied SNF requests for nursing home residents at a higher rate than MAOs that processed requests internally or other contractors

In August 2025, OIG reached out to five of the largest MAOs to request additional information about their reviews of SNF admission requests from nursing home residents. All of the MAOs reported that they use the same Medicare coverage rules to review all SNF requests. However, three of the MAOs reported that their contractor naviHealth had supplemental guidance for reviewers on how to review requests for nursing home residents. The guidance instructed reviewers to consider the reasons that the enrollee lives in a nursing facility. According to the guidance, those reasons frequently impact a patient’s ability to meaningfully participate in daily skilled therapy. Our analysis showed that in June 2024, naviHealth denied SNF requests for nursing home residents at a higher rate than other contractors or MAOs that processed requests internally.



Source: OIG analysis of June 2024 MAO SNF prior authorization data and 2024 data from the Minimum Data Set.

CONCLUSION AND RECOMMENDATIONS

More than half of people with Medicare were enrolled in Medicare Advantage in 2024, and MAOs received an estimated \$494 billion to cover at least the same services as original Medicare for these enrollees. MAOs are expected to implement critical program controls to avoid unnecessary costs and ensure program integrity while at the same time ensuring that enrollees can get necessary and appropriate care. However, our findings show patterns of denial and appeal outcomes for SNF-level care that raise concerns.

The high concentration of enrollees in a few large MAOs, and the use of contractors to process prior authorization requests, means that the policies and performance of a few companies can impact care for millions of people. Overall, MAOs denied 12 percent of requests for SNF admission in June 2024 (approximately 13,500 of 109,400 requests). However, across MAOs, people living in nursing homes were denied SNF-level care 40 percent of the time. When enrollees appealed (18 percent of SNF denials), MAOs nearly always overturned the denial, particularly among the larger MAOs and MAO contractors. The high overturn rates raise concerns that some enrollees may have been inappropriately denied access to SNF-level care at their first request. These findings warrant further scrutiny from CMS.

As enrollment in Medicare Advantage continues to grow, so does the urgency and importance of ensuring that MAOs are delivering on the value that the Federal Government pays them to provide. Given OIG's body of work that has raised concerns about enrollee access to care in Medicare Advantage, CMS should collect more comprehensive data to efficiently and effectively identify MAOs with problematic patterns of denials and take action as appropriate.³⁵ In our companion report, OEI 09-24-00330, we made a recommendation that is also relevant to addressing the issues raised in this report. Specifically, we recommended that CMS:

Regularly collect request-level prior authorization data that include standardized service type and contractor information.³⁶

This will enable analysis of prior authorization data by service type, such as the analysis that we conducted in this report.

Based on the findings of this report, we further recommend that CMS:

Take action to address any breakdowns in the initial reviews of SNF admission requests that are driving the extremely high overturn rate of SNF admission denials

Given the extremely high overturn rate of SNF denials when enrollees appeal, CMS should engage with MAOs to identify the reasons for the high overturn rate and take

corrective action as appropriate. High overturn rates when enrollees appeal could indicate problems with the MAO's or contractor's initial review process, such as misinterpretation or misapplication of CMS coverage rules. High overturn rates could also indicate that providers are not submitting all needed documentation at the first request and are adding documentation with appeal submissions. Whatever the reasons for the high overturn rates, action is needed to ensure that MAOs and contractors have the documentation and processes that they need to make the correct decision at the initial request and that enrollees receive needed services without the delay and added burden for all parties of going through the appeals process.

Assess the reasons for variation in SNF denial rates across MAOs and contractors and take action as appropriate

CMS should identify the reasons for the variation in prior authorization denials for SNF admission and, if needed, take appropriate steps to address the differences. Variation in denial rates could indicate differences in program integrity efforts or misinterpretation or misapplication of CMS coverage rules. Extremely high denial rates may indicate inappropriate denials.

For example, CMS could engage with the MAOs that we identified with high denial rates for SNF admission to determine whether they are meeting program requirements and take corrective action as appropriate. CMS could also engage with MAOs that employ contractors with high denial rates to determine whether the contractors are receiving appropriate training, guidance, and oversight from MAOs to comply with Medicare coverage rules.

Assess reasons for the differences in SNF denial rates between nursing home residents and other enrollees and take action as warranted

CMS should examine the reasons why MAOs are denying access to SNF-level care for nursing home residents at nearly four times the rate of nonresidents and, if needed, take appropriate steps to address the difference. To do this, CMS could conduct targeted case file reviews or focus groups with MAOs and SNFs. CMS should take appropriate steps to clarify for MAOs, SNFs, hospitals, and Medicare Advantage enrollees whether and how nursing home residency affects an enrollee's ability to meet coverage criteria for SNF admission.

AGENCY COMMENTS AND OIG RESPONSE

CMS did not explicitly concur or nonconcur with our three recommendations. CMS stated that it uses several oversight tools, including annual audits, to ensure that the Medicare Advantage program provides adequate health care access to enrollees.

Regarding our first recommendation—to take action to address any breakdowns in the initial reviews of SNF admission requests that are driving the extremely high overturn rate of SNF admission denials—CMS stated that, through its annual audits, it will continue to evaluate whether MAOs applied clinical criteria in accordance with CMS rules and appropriately adjudicated prior authorization determinations. However, CMS did not indicate whether and how it would target SNF denials in future audits or other oversight efforts. We ask that CMS include any specific plans or actions to address breakdowns in the initial reviews of SNF admission requests in its Final Management Decision.

Regarding our second recommendation—to assess the reasons for variation in SNF denial rates across MAOs and contractors and take action as appropriate—CMS stated that it has launched a Medicare Advantage data collection pilot. Plans that have volunteered to join the pilot will report service-level prior authorization data, and CMS will analyze the data, including denial rates by service type and third-party vendors (i.e., contractors). However, it is unclear how CMS could recalculate SNF denial rates across MAOs and contractors through the data collection pilot because it does not include the data needed to conduct such analyses (i.e., standardized service type data on requests for SNF admissions and contractor names). In its Final Management Decision, we ask that CMS specify what steps it has taken or is planning to assess the reasons for variation in SNF denial rates across MAOs and contractors that we identified in this report.

Regarding our third recommendation—to assess reasons for the differences in SNF denial rates between nursing home residents and other enrollees and take action as warranted—CMS stated that it can target denials of SNF services as part of its program audits and would consider our findings when deciding next steps. However, CMS did not indicate whether and how it would target SNF denials for nursing home residents in future audits or other oversight efforts. We ask that CMS include any specific plans or actions to assess reasons for these different SNF denial rates in its Final Management Decision.

For the full text of CMS's comments, see Appendix F.

METHODOLOGY

MAO Selection

To select the MAOs for this review, we used CMS Part C (i.e., Medicare Advantage) enrollment data to identify the largest MAO parent companies by enrollment. We selected the 19 parent companies that had at least 200,000 people enrolled in any Medicare Advantage contract in June 2024. These 19 parent companies operated contracts in 55 States and territories and covered 29.3 million people (see Appendix A). The companies covered approximately 86 percent of total Medicare Advantage enrollment in June 2024.

Data Collection

From each of the 19 selected MAOs, we collected a list of all prior authorization requests for admission to SNFs that the MAOs or their contractors processed during June 2024. For each prior authorization request, the MAOs provided detailed information, including:

- Enrollee identification numbers for enrollees who requested the service
- An open text description of the service requested
- A service type label (SNF)
- The outcome of the initial request (approved or denied)
- The name of the MAO contractor that processed the request (if applicable)
- Whether the request was appealed and the appeal outcome
- Key dates (e.g., date of request, initial decision, appeal)

Where appropriate, we followed up with MAOs to resolve discrepancies in the submitted data.

In addition to collecting data from MAOs, we also analyzed data from the Medicare Enrollment Database, CMS's Health Plan Management System, and CMS's Minimum Data Set.

Analysis

Denial rates. We calculated the number and rate at which the selected MAOs denied prior authorization requests for SNF admission in June 2024 (see Appendix B). First, we summed all denials of requests for SNF admission across the 19 MAOs. To calculate the denial rate, we divided the total number of denials by the total number of prior authorization decisions that MAOs issued for SNF admission in June 2024.³⁷

We also calculated these rates by MAO to examine the distribution of rates across MAOs and to identify MAOs with particularly high rates. For the overall and MAO-level analyses, we included both requests processed internally by the MAOs and requests submitted to the MAOs that were processed by their contractor(s), if applicable (see Appendix B).

Tax status. To examine denial rates by tax status for each setting, we used data from CMS's Health Plan Management system to categorize contracts as for-profit or nonprofit/not-for-profit and then determined differences in denial rates (see Appendix E).

Appeals. To calculate the appeal rate, we divided the total number of appeals by the total number of initial denials issued across all MAOs. To calculate the overturn rates, we summed the total number of approved appeal decisions and divided by the total number of approved or denied appeal decisions. We calculated overturn rates by MAO for MAOs that processed at least 50 appeals (see Appendix B).

Timelines. To examine the typical timelines for requests, we used the following date fields: initial request date, initial request decision date, appeal date (if applicable), and appeal decision date (if applicable). Because outliers can affect the average (mean), we used the median (middle) number of days to represent the typical wait time between dates.

MAO contractors. To examine the denial and appeal outcome rates by MAO contractor, we calculated the rates by contractor across any MAO that they processed requests for. We compared the contractor rates with the rates for requests that were processed internally by MAOs (see Appendix C).

Nursing home resident analysis. To examine the SNF admission denial rate for nursing homes residents, we used 2024 Minimum Data Set assessment data for the enrollees in our population. We followed the methods described in CMS's *MDS 3.0 Quality Measures User's Manual* to group assessments into episodes and to identify nursing home residents.³⁸ We identified nursing home residents as those with a "long-stay" episode (i.e., where the cumulative length of time in the facility for that episode was 101 or more days). To account for patients who may have left the nursing homes for extended hospitalization, we included patients with long-stays that ended up to 2 months before our June 2024 timeframe (i.e., long-stays that ended after April 1, 2024). We calculated denial rates for enrollees who were classified as long-stay and compared that rate to the denial rate for all other enrollees (see Appendix D).

To better understand the difference in denial rates for nursing home residents compared to other enrollees, we followed up in writing with five of the larger MAOs. To hear perspectives from patient advocacy and industry organizations, we interviewed representatives from the Center for Medicare Advocacy, LeadingAge, and the American Healthcare Association.

Dual eligibility analysis. To examine the SNF admission denial rate by dually eligible enrollees and Medicare Advantage-only enrollees, we used data from the Medicare Enrollment Database to categorize enrollees by whether they were dually eligible for Medicare and Medicaid. We then determined differences in denial rates by dually eligible and not dually eligible (i.e., Medicare Advantage-only) (see Appendix D). See our companion report, OEI-09-24-00330, for differences in rates for LTCH and IRF admission requests by enrollee characteristics.

Limitations

This report analyzed data provided by the selected MAOs. We followed up with MAOs to resolve discrepancies in the submitted data, but we did not review medical records to independently verify the accuracy of the data.

We also analyzed Medicare data and Minimum Data Set data on nursing home residents. We did not independently verify the accuracy of these data for this study.

Standards

We conducted this study in accordance with the *Quality Standards for Inspection and Evaluation* issued in 2020 by the Council of the Inspectors General on Integrity and Efficiency.

APPENDICES

Appendix A: Enrollment in the Sampled MAOs, June 2024

The 19 MAOs included in this review enrolled between 219,000 and 9.5 million people in June 2024. Service areas for these MAOs covered all 50 States, the District of Columbia, and 4 territories (Guam, the Northern Mariana Islands, Puerto Rico, and the U.S. Virgin Islands).

MAO Parent Organization	Enrollment	Number of States and/or Territories in Service Area	Number of Nonprofit Contracts	Number of For-Profit Contracts
UnitedHealth Group, Inc.	9,489,175	55	0	64
Humana Inc.	6,167,833	55	0	42
CVS Health Corporation	4,275,902	55	0	41
Elevance Health, Inc.	2,029,692	55	1	41
Kaiser Foundation Health Plan, Inc.	1,908,557	9	7	1
Centene Corporation	1,140,398	37	1	85
BCBS of Michigan Mutual Ins. Co.	709,396	55	2	4
The Cigna Group	593,784	55	0	14
Highmark Health	421,249	55	7	0
GuideWell Mutual Holding Corporation	333,565	52	1	5
Healthfirst, Inc.	297,333	51	3	1
MHH Healthcare, L.P.	290,618	2	0	2
SCAN Group	277,975	5	6	0
Molina Healthcare, Inc.	253,749	16	0	18
Corewell Health	249,561	51	3	0
Aware Integrated, Inc.	227,548	55	2	0
Health Care Service Corporation	224,292	55	4	5
Devoted Health, Inc.	223,423	13	0	24
UPMC Health System	219,213	54	2	2
Total	29,333,263	-	39	349

Source: OIG analysis of CMS Medicare Advantage contract and enrollment data for June 2024.

Appendix B: Volumes and Rates of SNF Prior Authorization Requests and Appeals by MAO, June 2024

MAO	Initial Approvals	Initial Denials	Denial Rate	Appeals Overturned	Appeals Upheld	Overturn Rate
Aware Integrated, Inc.	620	28	4.3%	1	0	-
BCBS of Michigan Mutual Ins. Co.	2,336	241	9.4%	68	7	90.7%
Centene Corporation	2,459	291	10.6%	33	2	-
The Cigna Group	2,466	239	8.8%	48	2	96.0%
Corewell Health	658	27	3.9%	0	0	-
CVS Health Corporation	13,938	2,170	13.5%	375	7	98.2%
Devoted Health, Inc.	793	99	11.1%	6	24	-
Elevance Health, Inc.	6,475	786	10.8%	93	0	100%
GuideWell Mutual Holding Corporation	553	30	5.1%	6	1	-
Health Care Service Corporation	695	50	6.7%	2	0	-
Healthfirst, Inc.	723	25	3.3%	3	0	-
Highmark Health	1,754	168	8.7%	12	6	-
Humana Inc.	22,880	3,570	13.5%	619	53	92.1%
Kaiser Foundation Health Plan, Inc.	1,298	212	14.0%	17	22	-
MHH Healthcare, L.P.	241	1	0.4%	0	0	-
Molina Healthcare, Inc.	453	138	23.4%	8	5	-
SCAN Group	253	4	1.6%	0	0	-
UnitedHealth Group, Inc.	36,124	5,362	12.9%	1,022	3	99.7%
UPMC Health System	1,210	17	1.4%	0	0	-
Total	95,929	13,458	12.3%	2,313	132	94.6%

Source: OIG analysis of June 2024 MAO SNF prior authorization data.

Note: SNF prior authorization denial rates were calculated only for MAOs that received at least 50 SNF requests; overturn rates were calculated only for MAOs that received at least 50 SNF appeals.

Appendix C: Volumes and Rates of SNF Prior Authorization Requests and Appeals by Contractor, June 2024

Reviewing Entity	Initial Approvals	Initial Denials	Denial Rate	Appeals Overturned by MAOs	Appeals Upheld by MAOs	Overturn Rate
naviHealth	47,285	7,891	14.3%	1,520	53	96.6%
MAOs – internal	31,345	3,867	11.0%	565	73	88.6%
Other contractors (excluding naviHealth)	17,299	1,700	8.9%	228	6	97.4%
Carelon	4,822	707	12.8%	91	0	100.0%
CareCentrix	1,786	216	10.8%	36	3	-
EviCore	3,131	279	8.2%	50	2	96.2%
WellMed	4,041	287	6.6%	44	0	-
Contractors not listed above	3,519	211	5.7%	7	1	-

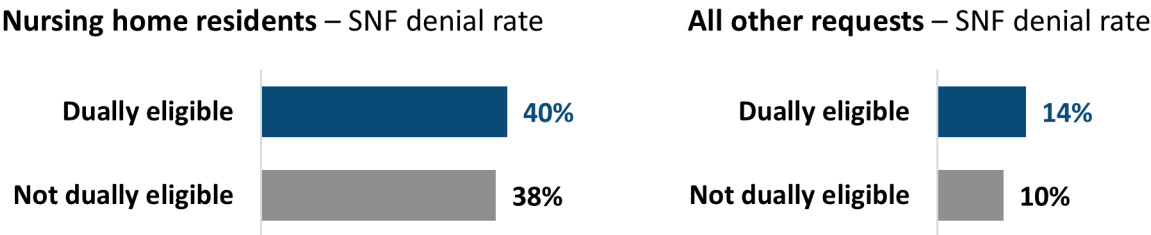
Source: OIG analysis of June 2024 SNF prior authorization data.

Note: Overturn rates were calculated only for reviewing entities that received at least 50 SNF appeals.

Appendix D: Volumes and Rates of SNF Prior Authorization Requests and Appeals by Enrollee Characteristics, June 2024

Enrollee Characteristic	Initial Approvals	Initial Denials	Denial Rate
Nursing home residents	1,965	1,283	39.5%
All other enrollees	93,964	12,175	11.5%
Enrollees eligible for both Medicare and Medicaid (“dually eligible”)	31,810	5,912	15.7%
Medicare Advantage-only enrollees	64,119	7,546	10.5%

Dually eligible enrollees were more likely to be denied SNF admission than enrollees who were not dually eligible, even after accounting for nursing home residency status



Source: OIG analysis of June 2024 SNF prior authorization and appeals data, enrollee demographic data from the Medicare Enrollment database, and 2024 data on enrollee nursing home stays from the Minimum Data Set.

Appendix E: Volumes and Rates of SNF Prior Authorization Requests and Appeals by MAO Contract Profit Status, June 2024

MAO Contract Profit Status	Initial Approvals	Initial Denials	Denial Rate
For-profit contracts	87,310	12,689	12.7%
Nonprofit/Not-for-profit	8,619	769	8.2%

Source: OIG analysis of June 2024 MAO SNF prior authorization data and 2024 contract tax status from CMS's Health Plan Management System.

Appendix F: Agency Comments

CMS's official comments begin on the next page.



Administrator

Washington, DC 20201

DATE: May 4, 2026

TO: Megan Tinker
Chief of Staff

FROM: Dr. Mehmet Oz 
Administrator
Centers for Medicare & Medicaid Services

SUBJECT: Office of Inspector General (OIG) Draft Report: Medicare Advantage Organizations Overturned Nearly All Appealed Prior Authorization Denials for Skilled Nursing Facility Admission, Raising Concerns About Initial Denials, OEI-09-24-00331

The Centers for Medicare & Medicaid Services (CMS) appreciates the opportunity to review and comment on the Office of Inspector General's (OIG) draft report. CMS is committed to program integrity and its enforcement and oversight of the Medicare Advantage (MA) program.

MA plans are Medicare-approved managed care plans offered by Medicare Advantage Organizations (MAOs), which are private companies, as an alternative to Original Medicare. MAOs must generally cover the same benefits as Original Medicare, such as stays in Skilled Nursing Facilities (SNF). However, as part of the managed care structure, MAOs may apply internal coverage policies that are no more restrictive than original Medicare's national and local coverage criteria that are based on current evidence in widely used treatment guidelines or clinical literature to make medical necessity determinations, as permitted in 42 C.F.R. § 422.101(b)(6).

For Medicare basic benefits, MAOs must make medical necessity determinations in accordance with all medical necessity determination requirements, outlined at 42 C.F.R. § 422.101(c)(1); based on the circumstances of each specific individual, including the patient's medical history, physician recommendations, and clinical notes; and in line with all fully established Traditional Medicare coverage criteria. This includes established criteria in applicable Medicare statutes, regulations, National Coverage Determinations (NCDs) or Local Coverage Determinations (LCDs).

CMS uses several oversight tools to ensure that the MA program provides adequate health care access to enrollees. For example, as part of its oversight strategy, CMS conducts audits of a subset of MAOs each year to assess compliance with the terms of their contracts with CMS, in particular the requirements associated with access to medical services, drugs, and other enrollee protections required by Medicare. The audit program operates on a continuous, rotating basis to promote broad coverage over time; therefore, the MAOs selected for audit may vary annually, and not all MAOs are audited in a given year. This approach allows CMS to be responsive and direct its audits to areas of identified concern, such as service types with a high rate of denial. As designed, these audits extend beyond data analysis by evaluating whether MAOs applied clinical criteria in accordance with CMS rules and appropriately adjudicated those determinations. In cases where denied claims are overturned on appeal, CMS notes the initial denial may still have been warranted if it stemmed from incomplete documentation that was

subsequently provided during the appeal. Following an audit, CMS notifies the plan of identified noncompliance, such as when the plan's denial of coverage does not align with CMS rules and may prevent enrollees from accessing care. MAOs are then required to submit corrective action plans when remediation is necessary to strengthen internal controls, prevent recurrence of noncompliance, and/or address any impact to enrollees. Further, plans that are found to have significant or repeated violations may be subject to penalties, such as Civil Monetary Penalties and intermediate sanctions (suspension of payment, enrollment, and/or marketing activities). In recent years, CMS has increased the transparency of noncompliance with CMS requirements by publishing Compliance and Enforcement actions on the CMS.gov website¹ and developing a publicly available annual report with business practices MAOs can adopt to continue improving performance.

Recently, CMS has developed a data collection pilot called the Service Level Data Collection for Initial Determination and Appeals.² CMS implemented this new data collection in February 2026 and will collect all service-level determinations, including prior authorization information across all settings, including SNFs. Pilot plans will report whether a third-party vendor participated, in any capacity, in the determination review or decision-making related to determination reviews and decision-making processes (which include prior authorization requests). This data will permit CMS to analyze the relationship between denial rates and whether a third-party vendor was involved with the plans participating in the pilot. This data will also allow CMS to identify and analyze prior authorization decisions by service type for pilot participants and whether there was vendor involvement.

OIG's recommendations and CMS's responses are below.

OIG Recommendation

CMS should take action to address any breakdowns in the initial reviews of SNF admission requests that are driving the extremely high overturn rate of SNF admission denials.

CMS Response

As mentioned above, CMS's broad oversight strategy includes auditing a varying selection of MAOs each year to assess their compliance with contract terms, particularly regarding access to medical services, drugs, and other enrollee protections required by Medicare. During these audits, CMS will continue to evaluate whether MAOs applied clinical criteria in accordance with CMS rules and appropriately adjudicated those determinations. CMS will also continue requiring MAOs to submit corrective action plans when remediation is necessary to strengthen internal controls, prevent recurrence of noncompliance, and/or address any impact to enrollees.

OIG Recommendation

CMS should assess the reasons for variation in SNF denial rates across MAOs and contractors and take action as appropriate.

CMS Response

As mentioned above, CMS has launched a data collection pilot, in which MA plans volunteered to join. Among other data, pilot participants will be reporting service-level prior authorization data, including service types, across all settings. Pilot participants will also report information related to whether a third-party vendor was involved in the determination review and decision-making process. CMS will analyze

¹ Please see: <https://www.cms.gov/medicare/audits-compliance/part-c-d/part-c-and-part-d-enforcement-actions>

² Please see December HPMS memo here: <https://www.cms.gov/about-cms/information-systems/hpms/hpms-memos-archive-weekly/hpms-memos-wk-3-december-15-19>.

pilot denial rates by a number of characteristics, including by service and involvement of a third-party vendor. CMS looks forward to gathering lessons learned from conducting this pilot, which will inform our next steps.

OIG Recommendation

CMS should assess reasons for the differences in SNF denial rates between nursing home residents and other enrollees and take action as warranted.

CMS Response

Under the Part C audit framework, CMS can already target denials of SNF services for review. CMS will consider these findings when deciding next steps if audits confirm any link between denial rates and enrollee setting.

CMS thanks OIG for their efforts on this issue and looks forward to working with OIG on this and other issues in the future.

ENDNOTES

¹ Meredith Freed et al., “[Medicare Advantage in 2024: Enrollment Update and Key Trends](#)” (Aug. 8, 2024). Accessed on Dec. 1, 2025.

² MedPAC, [Report to the Congress: Medicare Payment Policy, Chapter 11: The Medicare Advantage program: Status report](#), March 2025. Accessed on Oct. 20, 2025.

³ At a minimum, MAOs must cover the same services as in original Medicare, although they may also offer supplemental benefits. 42 CFR §§ 422.101(a) and (b); 422.102. MAOs are not responsible for paying hospice care costs for beneficiaries; original Medicare pays these costs.

⁴ HHS, [2024 Medicare Fee-for-Service Supplemental Improper Payment Data](#). Accessed on Aug. 18, 2025.

⁵ 5 CFR § 422.304.

⁶ American Hospital Association, “[Fact Sheet: Post-acute Care](#)” (July 2019). Accessed on Dec. 1, 2025.

⁷ OIG interviews with post-acute care providers, industry groups, and hospitals, 2024.

⁸ CMS, “[What are Long-Term Care Hospitals?](#)” (June 2019). Accessed on Dec. 1, 2025.

⁹ CMS, “[Inpatient Rehabilitation Hospitals & Inpatient Rehabilitation Units](#)” (November 2025). Accessed on Jan. 6, 2026.

¹⁰ MedPAC, [Report to the Congress: Medicare Payment Policy, Chapter 8: Skilled nursing facility services](#), March 2018. Accessed on Jan. 6, 2026.

¹¹ Although there are some circumstances in which a patient may receive care at an LTCH, IRF, or SNF without a preceding acute-care hospital stay, for the purposes of this study, we refer to these facilities collectively as “post-acute care facilities.”

¹² MAOs are responsible for complying with coverage requirements even if they contract with another company to conduct reviews. 42 CFR § 422.504(i)(1).

¹³ 42 CFR § 422.578.

¹⁴ MedPAC, [Report to the Congress: Medicare and the Health Care Delivery System, Chapter 5: Medicare beneficiaries in nursing homes](#), June 2025. Accessed on Dec. 1, 2025.

¹⁵ OIG, [Some Medicare Advantage Organization Denials of Prior Authorization Requests Raise Concerns About Beneficiary Access to Medically Necessary Care \(OEI-09-18-00260\)](#), Apr. 27, 2022.

¹⁶ OIG, [Medicare Advantage Appeal Outcomes and Audit Findings Raise Concerns About Service and Payment Denials \(OEI-09-16-00410\)](#), Sept. 25, 2018.

¹⁷ OIG, [High Rates of Prior Authorization Denials by Some Plans and Limited State Oversight Raise Concerns About Access to Care in Medicaid Managed Care \(OEI-09-19-00350\)](#), July 17, 2023.

¹⁸ OIG, [*Keystone First Should Improve Its Procedures for Reviewing Service Requests That Require Prior Authorization \(A-03-20-00201\)*](#), Dec. 20, 2022.

¹⁹ OIG, [*New York Did Not Ensure That a Managed Care Organization Complied With Requirements for Denying Prior Authorization Requests \(A-02-21-01016\)*](#), Sept. 18, 2023.

²⁰ OIG, [*Some Medicare Part D Beneficiaries Face Avoidable Extra Steps That Can Delay or Prevent Access to Prescribed Drugs \(OEI-09-16-00411\)*](#), Sept. 19, 2019.

²¹ CMS, Medicare Advantage and Part D Enrollment Data, [*Monthly Enrollment by Contract*](#), June 2024. Accessed on Jan. 30, 2025.

²² OIG, *Medicare Advantage Organization's Use of Prior Authorization for Post-Acute Care*, OEI-09-24-00332, expected to be issued in FY 2027.

²³ MedPAC, [*Report to the Congress: Medicare Payment Policy, Chapter 6: Skilled nursing facility services*](#), March 2025, p. 8. Accessed on Dec. 1, 2025. Original Medicare paid \$25 billion for 1,583,000 SNF stays in 2023, for an approximate average of \$16,000 per stay.

²⁴ MedPAC, [*Report to the Congress: Medicare Payment Policy, Chapter 7: Home health care services*](#), March 2025, p. 10. Accessed on Dec. 1, 2025.

²⁵ NORC at the University of Chicago, [*Analysis of Hospital Discharges to PAC Settings Among Medicare Beneficiaries*](#), June 2025. Accessed on July 11, 2025.

²⁶ A survey of hospitalists found that post-acute care issues, including rejections of placements, was the most common cause of delays in care that would likely lead to additional days in the hospital (i.e., “avoidable days”). See Sajan Patel, Sasha Morduchowicz, and Nader Najafi, [*“Avoidable Days: Identifying and Quantifying Delays in Inpatient Care,” Journal of Hospital Medicine*](#) (May 2017). Accessed on Dec. 15, 2025.

²⁷ Testimony of David Basel, Vice President of Clinical Quality and Population Health Officer, Avera, before the United States House of Representatives Committee on Ways and Means Subcommittee on Health and Subcommittee on Oversight, [*Medicare Advantage: Past Lessons, Present Insights, Future Opportunities*](#) (July 22, 2025). Accessed on Dec. 1, 2025.

²⁸ Kanishk Aggarwal et al., [*“Exploring the Ramifications of Delayed Hospital Discharges: Impacts on Patients, Physicians, and Healthcare Systems,” Cureus*](#), vol. 16, no. 5 (May 28, 2024). Accessed on Dec. 15, 2025.

²⁹ For the purposes of this report, we refer to any entity that performs tasks on behalf of an MAO as an “MAO contractor.” These entities may include “first tier” entities, which enter into a written arrangement with an MAO to provide administrative services or health care services; “downstream” entities, which enter into an arrangement below the level of the arrangement between an MAO and a first tier entity; and “related” entities, which are related to the MAO by common ownership or control and perform some of the MA organization’s functions under contract or delegation. The MAO is responsible for assuring that its contractors and any downstream contractors have the information necessary to know how to comply with the requirements under the Medicare Advantage program. CMS, [*Medicare Managed Care Manual, Chapter 11: Medicare Advantage Application Procedures and Contract Requirements*](#), Apr. 25, 2007. Accessed on Dec. 1, 2025.

³⁰ UnitedHealth Group, Inc., reported that “naviHealth is an indirect wholly owned subsidiary of UnitedHealth Group . . . naviHealth is a first tier downstream contractor and delegate of various UnitedHealthcare entities and also an affiliated entity through common indirect ownership by UnitedHealth Group.”

³¹ OIG, *The Three Largest Medicare Advantage Organizations Denied Requests for Long-Term Acute Care and Inpatient Rehabilitation at Some of the Highest Rates*, OEI-09-24-00330, June 11, 2026.

³² For the purposes of this report, we defined “nursing home residents” as those who had a nursing home episode of at least 101 cumulative days in a nursing facility that started before their SNF admission request and ended any time after April 1, 2024. This aligns with the definition of “long-stay” episodes in [CMS’s MDS 3.0 Quality Measures User’s Manual](#), Version 17. Accessed on Dec. 1, 2025.

³³ MAOs may cover intermittent skilled nursing services and/or outpatient therapy in a provider’s office, a long-term care nursing facility, or in a patient’s home through home health services.

³⁴ In order to meet the coverage criteria for admission to a SNF, enrollees must, among other requirements, need daily skilled care (e.g., as intravenous fluids/medications or physical therapy), that is provided by or under the supervision of skilled nursing or therapy staff, to improve or maintain the enrollee’s current condition or to prevent or delay it from getting worse. CMS, “[Skilled nursing facility care](#).” Accessed on Sept. 30, 2025.

³⁵ At the time of our review, CMS did not regularly collect detailed, request-level prior authorization data from all MAOs. In December 2025, CMS solicited MAOs to volunteer for a prior authorization data collection pilot and published the draft requirements for the data that it will be collecting. The prior authorization data that CMS plans to collect do not include a standardized service type variable for all service types or the name of the contractor that processed the request (if applicable). CMS, Service Level Data Collection for Initial Determinations and Appeals - Pilot Participation, Dec.16, 2025. Available at: <https://www.cms.gov/about-cms/information-systems/hpms/hpms-memos-archive-weekly/hpms-memos-wk-3-december-15-19>.

³⁶ OIG, *The Three Largest Medicare Advantage Organizations Denied Requests for Long-Term Acute Care and Inpatient Rehabilitation at Some of the Highest Rates*, OEI-09-24-00330, June 11, 2026.

³⁷ We asked MAOs to report data based on the date that the initial request decisions were issued. This means, for example, that a request that an MAO received at the end of June but had not yet issued a decision for by June 30, 2024, would not be included in our data. Similarly, decisions issued by MAOs at the beginning of June 2024 may include decisions for requests that were received in May.

³⁸ CMS, [MDS 3.0 Quality Measures User’s Manual, Version 17.0, Chapters 1 and 4](#), January 2025. Accessed on Dec. 1, 2025.

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